

Mohr Insurance — Dynamic Sales Script

Script version 5 · branched reference & agent worksheet · four tracks, each with its own questions · umbrella priced from the needs assessment

Standing compliance rule

BEFORE ANYTHING ELSE

A client who is 65 or older in California cannot be sold cancer / heart attack / stroke cover or skilled nursing cover. Because neither can be sold, **do not embed either exposure** — skip the major-exposures set-up, the CHS conversation and the skilled nursing conversation entirely.

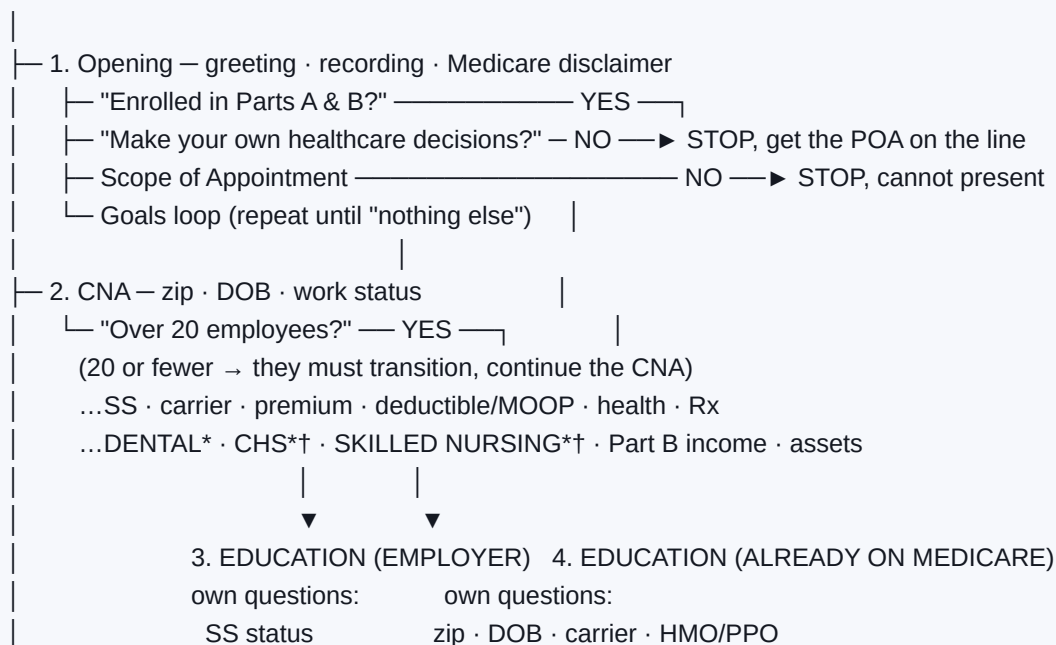
Dental can still be sold. Ask whether it is important to them as normal; it still prices \$50 into the Medicare Supplement umbrella. That leaves the Advantage umbrella at \$0, since an Advantage plan already bundles dental.

Gold / Silver / Bronze is cancer, heart attack and stroke cover, so it cannot be sold either. On the employer track there is no Medicare plan to offer in its place, so that call ends after the delay recommendation and the CMS L564 education.

Their zip code and date of birth decide all of this, and both are captured early on every track.

How this script branches

EVERY CALL



carrier · HMO/PPO premium · max OOP · hit it
 premium · max OOP DENTAL* · CHS* · SKILLED NURSING*
 hit it → commitment ↓
 ↓ "Want the Supp vs MA rundown?"
 Rec 1: delay Medicare |— YES —► education → pricing
 (branches on SS) |— NO —► straight to pricing
 Rec 2: Gold/Silver/Bronze ↓
 ↓ 3 STEP CLOSE
 close (umbrella · lock in plan · effective date)

5. MEDICARE EDUCATION — T65 only

Scenario 1 on SS (auto-enrolled) / Scenario 2 not on SS (manual)

- Original Medicare → 20% exposure → paper check
- 6. Supplement → 7. Advantage → 8. Pricing with umbrella
- leaning → formal recommendation
- Scenario A (not on SS) / Scenario B (on SS)
- underwriting urgency → address → objections + pre-emptive close

* umbrella drivers † skipped entirely when the client is 65+ in California

Track	How you land on it	Its own questions	Close
Employer coverage	Not on Medicare · still working · over 20 employees . Exits at the work-status question.	SS · carrier · HMO/PPO · premium · max OOP · hit it	Gold / Silver / Bronze, then address
Already on Medicare	Parts A & B active. Exits right after the goals loop, before the CNA.	zip · DOB · carrier · HMO/PPO · premium · max OOP · hit it · dental · CHS · skilled nursing	3 step close
T65 — not on SS	Not on Medicare, not drawing Social Security	The full needs assessment in section 2	Scenario A
T65 — on SS	Not on Medicare, drawing Social Security	The full needs assessment in section 2	Scenario B

*An employer plan with **20 or fewer employees** makes Medicare the primary payer — the script says outright that those clients **must transition to Medicare**, so they stay in the needs assessment and run the T65 track.*

Umbrella pricing — driven by three CNA answers

What they told you in the CNA	Medicare Supplement	Medicare Advantage
Cancer / heart attack / stroke is a concern	+\$50	+\$50
Skilled nursing is a concern	+\$50	+\$100
Dental is important to them	+\$50	included with the plan
Maximum umbrella	\$150	\$150
California, 65 or older — cancer and skilled nursing removed, dental only	\$0 or \$50	\$0

Quote	Formula
Medicare Supplement	Plan G premium + Part D ≈ \$10 + umbrella (\$50 / \$100 / \$150) = one monthly price
Medicare Advantage	MAPD premium (usually \$0) + umbrella (\$50 / \$100 / \$150) = one monthly price

Quote the single all-in number. Do not itemize it out loud unless they ask.

65 or older in California: only the dental component survives. The Supplement umbrella is \$50 if dental matters to them and \$0 if it does not; the Advantage umbrella is \$0 either way. When the umbrella lands at \$0 there is nothing to sell today, and the close drops to two steps.

1 · Opening

Greeting and webinar confirmation

"Hey [Name], this is [Agent name] with Mohr Insurance Services calling for our scheduled Medicare appointment... how are you today?"

Wait for their response.

"Thank you for booking the call today. Hopefully I'll be able to help you out and answer any questions."

Call recording disclosure

"Now [Name], before we continue, I have to read these compliance disclosures before we dive in. You know we take compliance very seriously, so I need to let you know that **this call is being recorded for quality and compliance purposes.**"

Required Medicare disclaimer

"We do not offer every plan available in your area. Any information I provide is limited to the plans we represent. You can always visit Medicare.gov or call 1-800-MEDICARE to review all available options."

"Now, almost done here!"

Required verbatim. Do not paraphrase or skip.

Eligibility check branch point

"Before we go further, are you currently enrolled in Medicare Parts A and B?"

IF YES

They are **Avatar 4 — Currently on Medicare**. They will get the current plan review instead of an enrollment paragraph, and Scenario C at the close.

"And do you personally make your own healthcare decisions?"

IF NO

Stop. You cannot present or enroll without the authorized representative or power of attorney on the line. Ask who handles their healthcare decisions and get that person on the call, or reschedule with them present.

Scope of Appointment permission

"Now for today's call, we may discuss: Medicare Advantage Plans · Medicare Supplement Plans · Prescription Drug Plans · Ancillary Products. Do I have your permission to discuss these options with you today?"

*Wait for a **clear verbal YES**.*

IF NO

Do not proceed. Without a Scope of Appointment on the recording you cannot discuss any of the four product categories.

Transition into the CNA — the goals loop

"Perfect. I like to start with what's called a Client Needs Assessment. This helps me gather the right information and make sure I'm steering you in the best direction. But before we get started, I always like to ask: what would you like to accomplish today?"

Write down what they say — their words.

1.

2.

3.

"Perfect, so what I'm hearing is (repeat their goal). Does that sound about right?"

"Perfect, is there anything else you want to go over today?"

***Loop** until they have nothing else. You read every one of these back in the formal recommendation.*

PARTS A & B ALREADY ACTIVE

Skip the needs assessment below entirely and go to **section 4, Education (Already on Medicare)** — that section has its own questions.

2 · Client Needs Assessment

Zip code

"So for starters, what's your zip code?"

Repeat the zip code back. → "Awesome, got that down."

Zip:

Date of birth

"Now what is your date of birth?" → "Perfect, got that here."

DOB (mm/dd/yyyy): Age: Birth month:

Work status branch point

"Tell me a little bit about your current situation — are you still working, are you retired, what does that look like?"

IF STILL WORKING

"Does your employer have over 20 employees?"

Over 20 → **stop the needs assessment here** and go straight to section 3, Education (Employer). That section asks its own questions.

20 or fewer → they must transition to Medicare. Continue through the rest of the needs assessment below.

Work status:

Social Security status branch point

"Are you drawing Social Security yet?"

YES

Automatically enrolled into A & B · Part B deducted from their check · **Scenario B** at close.

NO

We enroll them manually · billed quarterly for the first quarter · **Scenario A** at close.

Current insurance carrier and source branch point

"Tell me a little bit about your current health insurance — is that through your employer or through the marketplace?"

"What carrier is that with?" · "And is that an HMO... PPO?"

Source: Carrier: HMO / PPO:

Monthly premium

"What are you paying for that right now on a month to month basis?"

\$ /mo

Deductible and max out of pocket

"And then you may not know this next part off the top of your head, but if you have your insurance card it should read off of there. What is your deductible and maximum out of pocket?"

Deductible: \$ Max out of pocket: \$

The max out of pocket is the entire basis of the employer-track pitch. Do not leave this blank.

"Have you hit that deductible or max out of pocket, and if so what happened?"

Health history

"And that leads me into my next question — sounds like you're pretty healthy / *have a little bit of a health history*. Is there anything with your recent health we should know about in the past 5 years?"

Pick the half of that sentence that matches what you've heard so far. Underwriting matters here.

"Okay, and are you taking any prescriptions right now, and what are they actively treating?"

Dental importance

umbrella driver

still asked in CA at 65+

"My next question would be, is dental insurance important to you, and do you currently have that with your employer or through the marketplace?"

IF YES

+\$50 to the Medicare Supplement umbrella. No effect on the Advantage umbrella — an Advantage plan already bundles dental.

Major exposures — the set-up

skipped in CA at 65+

"Now, in that webinar do you remember how we highlight major exposures?"

"Major exposures can really stem from any plan — whether it's your current plan with [carrier], or a Medicare Supplement or Advantage plan — but really in relation to two specific things."

Cancer, heart attack, stroke

umbrella driver

skipped in CA at 65+

"For starters — cancer, heart attack, stroke. Do we have any personal or family history with the big three?"

"I really appreciate you sharing that with me... and the reason we bring this up — I'll share a personal note. My grandmother had cancer three times, and she was on Medicare and had great coverage, but even with that coverage she had to pay **\$15,000** out of her own pocket for non-approved expenses. **Is something like that going to be a concern for you?**"

IF YES

+\$50 to both umbrellas.

Skilled nursing

umbrella driver

skipped in CA at 65+

"Now the next big exposure we see is skilled nursing. Do you have any personal or family history with that, and are you familiar with how that coverage functions with Medicare?"

"Medicare will help cover up to day 100 as long as you had a **3 day hospital stay** and are **showing signs of improvement**... but after day 100 you are responsible for all costs. And it's really important we focus on that 3

day hospital stay and showing signs of improvement."

"I had a client who was in skilled nursing for ten days and her insurance plan, Kaiser, said she was not improving and doing the exercises because she had a stroke — so they ended her care and she had to go home and pay **\$20,000** out of pocket for home care. So I'm assuming this will be a concern as well?"

IF YES

+\$50 to the Supplement umbrella · +\$100 to the Advantage umbrella (the Advantage side is buying skilled nursing coverage).

"And yeah, we'll come back to that in greater detail, but I really just wanted to make sure you're aware of these exposures."

Part B premium — income qualification

"Now, when you transition to Medicare you'll be responsible for a Part B premium — however that premium is going to be very dependent on your household income. Do you know where that's at at this moment in time?"

"And has that been pretty consistent over the last two years?" ... "And the reason I ask is they always look back two years prior."

Income: Consistent 2 yrs:

Retirement assets

"And another thing that can impact your Part B premium, which not a lot of people think about — do you have any retirement accounts that have over \$200,000 in qualified assets or more?"

"And the reason I ask is because when we plan to supplement our income with the money within those accounts, that can impact your Part B premium. Have you had the chance to talk to a retirement specialist yet?"

IF NO

"Okay, well we have a full partnership on hand, so if you ever want to sit down and have that conversation it is a service we offer **free of charge**, and we can look at getting you scheduled for that once we're done here — okay?"

Close out the CNA

"Awesome, that wraps up the needs assessment. Now we're going to walk through your options and answer the questions you had at the beginning."

3 · Education (Employer)

*Employer coverage with over 20 employees. This section has **its own questions** — the client left the needs assessment at the work-status question, so nothing below has been asked yet. This track ends here: no Supplement, no Advantage, no umbrella.*

Social Security status branch point

"Are you drawing Social Security yet?"

*If **yes**, take note — they will be automatically enrolled in Part A and must **request to delay Part B**. This changes two lines further down.*

Carrier and plan

"Tell me a little bit about your current health insurance — is that through your employer?"

"What carrier is that with?" · "And is that an HMO... PPO?"

Carrier: HMO / PPO:

Monthly premium

"What are you paying for that right now on a month to month basis?"

\$ /mo

Max out of pocket

"And then you may not know this next part off the top of your head, but if you have your insurance card it should read off of there. What is your maximum out of pocket?"

\$

"Have you hit that max out of pocket, and if so what happened?"

Get their commitment before you move on. This is the only exposure conversation on this track.

"Yeah, that makes sense. So I have all the information I need and have two recommendations for you."

Recommendation #1 — delay Medicare branches on SS

NOT DRAWING SOCIAL SECURITY

"So right now this transition to Medicare does not make much sense for you, so my first recommendation would be to **delay Part A and Part B entirely**. You have a strong plan with your employer and your premiums are pretty low."

DRAWING SOCIAL SECURITY

"So right now this transition to Medicare does not make much sense for you, so my first recommendation would be to **delay Part B entirely** — but you will be automatically enrolled in Part A, because you're drawing Social Security. You have a strong plan with your employer and your premiums are pretty low."

Peace of mind on penalties

"And I want to make sure you are at peace of mind with penalties, since that's always a big concern. **You won't be penalized for delaying Medicare** since you have creditable coverage with your employer. Are we clear on that?"

Wait for the response. Do not move on without a yes.

CMS L564 — the hall pass branches on SS

"Now when you do decide to retire, or you lose that creditable coverage, that's when you take action. That's when you take **CMS L564** — it's a form you'll take to your HR department and they'll help you fill it out. Then you'll take that form and you're gonna submit it along with your application for **[not on SS: Part A and Part B / on SS: Part B]**. That form is basically your **hall pass that voids all penalties**. Are we clear on that?"

CALIFORNIA, 65 OR OLDER — STOP HERE

Gold / Silver / Bronze is cancer, heart attack and stroke cover, so it cannot be sold, and there is no Medicare plan to offer in its place because they are staying on employer coverage. **End the call as education**. Answer their questions, make sure they know to call when they lose creditable coverage, and note when to follow up.

Recommendation #2 — cover the gaps

"Now my second recommendation would be to protect you from the gaps that employer plans typically leave behind."

STEP 1 — REITERATE THEIR EXPOSURE

"So right now, your employer plan has a max out of pocket of **[max OOP]**."

STEP 2 — EXPLAIN THE RISK

"What this means is, god forbid you have a bad health year, it's very likely you will pay **[max OOP]** for your healthcare **plus** potentially have loss of income — which are two significant exposures."

STEP 3 — PRESENT GOLD / SILVER / BRONZE

"That's where our **Gold, Silver and Bronze** options come in — they're designed to cover those big out-of-pocket risks while you stay on your employer plan."

"Now [first name], I want you to write down Gold, Silver and Bronze, and let me know once you've got that down."

*Wait for confirmation before **each** tier below. One at a time — this pace is the whole close.*

Tier	Cancer	Heart attack & stroke	Total benefit	Monthly
Gold	\$30,000	\$30,000	\$60,000	\$150
Silver	\$20,000	\$20,000	\$40,000	\$100
Bronze	\$10,000	\$10,000	\$20,000	\$50

"Awesome, so based on these prices, which option do you feel like you can comfortably afford on top of your employer coverage?"

Ask, then stop talking.

"Sounds good, so what would be a good address to send policy packets and information?"

*Handle objections and **always end with a pre-emptive close.***

4 · Education (Already on Medicare)

*Parts A & B are already active. This section has **its own questions** — the client left the opening right after the goals loop. No health history, no prescriptions, no Part B income, no retirement assets, and no Original Medicare walkthrough on this track.*

Zip code and date of birth

"So for starters, what's your zip code?"

Repeat the zip code back. → "Awesome, got that down."

"Now what is your date of birth?" → "Perfect, got that here."

Zip: State: DOB (mm/dd/yyyy): Age:

California and 65 or older? The umbrella comes off this call entirely — see the standing rule on page 1.

Carrier and plan

"Tell me a little bit about your current health insurance — what carrier is that with?" · "And is that an HMO... PPO?"

Carrier: HMO / PPO:

Monthly premium and max out of pocket

"What are you paying for that right now on a month to month basis?"

"And then you may not know this next part off the top of your head, but if you have your insurance card it should read off of there. What is your maximum out of pocket?"

"Have you hit that max out of pocket, and if so what happened?"

Premium: \$ Max OOP: \$

Dental umbrella driver still asked in CA at 65+

"And are you carrying any dental insurance right now?"

"Is that coverage important to you?"

IF IMPORTANT

+\$50 to the Medicare Supplement umbrella. No effect on the Advantage umbrella — an Advantage plan already bundles dental.

Major exposures — the set-up skipped in CA at 65+

"Gotcha. Now, in that webinar do you remember how we highlight major exposures?"

"Major exposures can really stem from any plan — whether it's your current plan with [carrier], or any other Medicare plan — but really in relation to two specific things."

Cancer, heart attack, stroke umbrella driver skipped in CA at 65+

"For starters — cancer, heart attack, stroke. Do we have any personal or family history with the big three?"

"I really appreciate you sharing that with me... and the reason we bring this up — I'll share a personal note. My grandmother had cancer three times, and she was on Medicare and had great coverage, but even with that coverage she had to pay **\$15,000** out of her own pocket for non-approved expenses. **Is something like that going to be a concern for you?**"

Get their commitment. *Not a shrug — a yes.*

IF YES

+\$50 to both umbrellas.

Skilled nursing umbrella driver skipped in CA at 65+

"Now the next big exposure we see is skilled nursing. Do you have any personal or family history with that, and are you familiar with how that coverage functions with Medicare?"

"Medicare will help cover up to day 100 as long as you had a **3 day hospital stay** and are **showing signs of improvement**... but after day 100 you are responsible for all costs. And it's really important we focus on that 3 day hospital stay and showing signs of improvement."

"I had a client who was in skilled nursing for ten days and her insurance plan, Kaiser, said she was not improving and doing the exercises because she had a stroke — so they ended her care and she had to go home

and pay **\$20,000** out of pocket for home care. So I'm assuming this will be a concern as well?"

Get their commitment.

IF YES

+\$50 to the Supplement umbrella · +\$100 to the Advantage umbrella.

"And yeah, we'll come back to that in greater detail, but I really just wanted to make sure you're aware of these exposures."

Do they want the rundown? branch point

"Now I have pretty much everything I need, and before we dive into pricing I always prefer to do a quick rundown of the difference between Medicare Advantage and Supplement — would you like that?"

YES

Run the Supplement and Advantage education (sections 5 and 6), then pricing.

NO

Skip both. Go straight to pricing.

Close for this track

3 STEP CLOSE — ALREADY ON MEDICARE

1. We get your umbrella coverage set up for **\$(X)** a month.
2. We review your current plan and lock in your new **[Supplement / Advantage]** plan.
3. Your new coverage kicks in on **[effective date]**.

California and 65 or older: step 1 is the dental umbrella if dental matters to them, and is removed altogether if it does not.

Then the underwriting urgency and the address question — same wording as the T65 track on the last page.

5 · Medicare Education — T65 only

How you get onto Medicare branch point

ON SOCIAL SECURITY

"So [name], what is going to happen is, since you are on Social Security, you are going to be **automatically enrolled** into Original Medicare Part A and Part B."

NOT ON SOCIAL SECURITY

"So [name], since you are not on Social Security, we will have to get you **manually enrolled** into Part A and Part B — and we will make this process as easy as possible for you. When the time comes, three months before your birth month, we will be sending you the instructions to get this done, and James has a very detailed video to help you with this step. It shouldn't take long — maybe about 10 minutes."

"Now, Original Medicare is what we call **the foundation of Medicare**. I believe James mentioned this in the webinar, but let me break it down for you again."

Introduction to Original Medicare

"**Part A** covers hospital expenses and is premium-free."

"**Part B** covers doctor visits and medical services and has a monthly premium of **\$202**."

IF ON SOCIAL SECURITY

"Since you're on Social Security, the Part B premium will be **deducted directly from your check**, so you won't need to worry about making separate payments."

IF NOT ON SOCIAL SECURITY

"Since you are not on Social Security, they are going to **bill you for the first quarter** for your Part B premium, and after that we can set up your banking info to do a monthly draft rather than paying it quarterly."

"Unfortunately, there's no way to avoid the Part B premium — it's just a standard Medicare requirement. Any questions so far?"

The exposure in Original Medicare alone

"Now, while Original Medicare is the foundation, we **never** recommend staying on it alone without additional coverage, because there's a lot of financial exposure with just Original Medicare."

"The biggest risk is the **20% coinsurance with no limit**. A \$100,000 medical bill means you're responsible for **\$20,000** out of pocket. And there is no cap on these costs, leaving you financially vulnerable."

"So to protect you, we'll discuss two options: a **Medicare Supplement plan**, also called Medigap, and **Medicare Advantage**, which is Part C. Before I go into the details, do you have any questions so far?"

"Do you have a piece of paper in front of you right now?"

Do not move on until they have pen and paper.

6 · Medicare Supplement education

"This is what we call the **Cadillac of health insurance**, because it provides the best coverage available. Now, it does come at a higher cost."

✓ COMPREHENSIVE COVERAGE

"After you meet a **\$283 annual deductible**, you are 100% covered for any medical costs. No matter the bill — \$100,000 or even a million dollars — you won't pay anything beyond that \$283 deductible. The reason it's called Medigap is because it covers the gaps that Original Medicare doesn't cover, that 20%. Does that make sense?"

Get confirmation before moving forward.

✓ NO NETWORK RESTRICTIONS

"You can see any doctor or hospital nationwide as long as they accept Medicare. So no concerns about being in or out of network."

✓ NO PRIOR AUTHORIZATION

"Medicare Supplement plans are federally regulated, meaning that by law they must cover anything Original Medicare does — so no insurance denials. If Medicare approves a procedure, your supplement plan automatically has to cover the rest."

THE GUARANTEED ISSUE WINDOW

"One thing to note before I finish up the Medicare Supplement plan is that you only have **one guaranteed issue period** to get a supplement plan, and that is when you first get onto Medicare. This means that if you elect not to go this route, if you want to switch in the future you will have to go through **medical underwriting** and can possibly be **denied** for that coverage later on."

"I know that's a lot of information, but do you have any questions on how a Medicare Supplement plan functions?"

7 · Medicare Advantage (Part C) education

"This is a more affordable option compared to the supplement plan, and it's structured differently."

✓ LOW MONTHLY COST

"Many plans have low monthly premiums, and it includes your medical, prescription drug coverage, and additional benefits like **dental, vision and hearing** at no extra cost."

△ PAY-AS-YOU-GO MODEL

"Unlike Medicare Supplement plans, you will pay co-pays and co-insurance for each service — doctor visits, hospital stays. Less predictable expenses if you have frequent medical needs."

"Think of Medicare Advantage as a **better version of your current health insurance coverage**. It essentially works the same way. I'm sure you have copays and coinsurance when you need medical care — Medicare Advantage works very similarly. The reason we say 9 times out of 10 it is a better version is because the plans usually have little to no deductible and very affordable copays."

△ NETWORK RESTRICTIONS

"You must use doctors and hospitals within your plan's network to get full coverage. You can go out of network, but expect higher costs."

"That's the general overview of Medicare Advantage — any questions on how a Medicare Advantage plan functions?"

8 • Pricing, recommendation and close

"Now that we've gone over the benefits of both plans, let's talk about cost."

Medicare Supplement (Medigap)	\$ / mo
Plan G premium	
Part D drug plan	≈ \$10
Umbrella (\$50 / \$100 / \$150)	
Total monthly — one price	

"Once you meet the small annual deductible of \$283 for the year, you owe **\$0** for covered medical services — so no surprise bills."

Medicare Advantage (Part C)	\$ / mo
MAPD premium (usually \$0)	
Umbrella (\$50 / \$100 / \$150)	
Total monthly — one price	

"You'll just have some small copays for doctor and specialist visits."

California and 65 or older: the Supplement umbrella line is dental only (\$50, or nothing if dental does not matter to them). Drop the umbrella line from the Advantage table entirely.

Which option are they leaning toward?

"Now that we've gone through both options and the detailed pricing breakdown, which route do you feel yourself leaning toward as of now?"

PAUSE. *Let them answer.*

"Okay, so you're leaning toward [Option 1 / 2]?"

Get their confirmation again.

Formal recommendation

"Based on everything you shared today — *(read back your notes from the call)* — my formal recommendation would be the **[Supplement / Advantage] plan**, with umbrella coverage at **\$(X)** a month, for about **\$(total)** a month all in."

PAUSE. The first one to talk loses.

The 3 simple steps branch point

"Do you still have that piece of paper? Perfect — I want you to write this down, because this is how we will get everything done, in these 3 simple steps."

SCENARIO A — T65, NOT ON SOCIAL SECURITY

1. We get your umbrella coverage set up for **\$(X)** a month.
2. We manually enroll you into Original Medicare Parts A & B three months before your birth month — it takes about 10 minutes.
3. We enroll you into your **[Supplement — 6 months / Advantage — 3 months]** before your 65th birthday.

SCENARIO B — T65, ON SOCIAL SECURITY

1. We get your umbrella coverage set up for **\$(X)** a month.
2. Since you're on Social Security you'll be automatically enrolled into Parts A & B — we just confirm that's all set.
3. We enroll you into your **[Supplement — 6 months / Advantage — 3 months]** before your 65th birthday.

Clients already on Medicare use the 3 step close in section 4 instead.

California and 65 or older with no dental sale: step 1 is removed and this becomes a two-step close. If dental matters to them, step 1 is the \$50 dental umbrella on the Supplement side.

"Step 1 is the only thing we need to knock out today."

Why we do this today

"The reason we want to go ahead and handle this now is because we need to make sure we can get you **approved through underwriting**. That way, if for some reason one company doesn't approve of you, we still have time to pivot and find another option before your effective date."

"The other important part is once you're approved, this **locks in your rate based on your age right now** — so it's never going to jump because of your health later on."

"I'll handle the entire enrollment process for you. It's only a few quick health questions and we should be able to get everything wrapped up in the next few minutes. Now let me get everything pulled up for us."

"And what was going to be a good street address for you, so I make sure you receive your packages and cards?"

*Move straight into the application. Don't ask permission — assume it. Then handle objections and **always end with a pre-emptive close**.*

Mohr Insurance Services · Dynamic sales script reference, script version 5. The interactive version routes the avatar and prices the umbrella for you — this document is the offline backup.